

1. Flood the injured part with cold water for at least ten minutes or until the pain is relieved. If water is not available, any cold, harmless liquid, such as milk, can be used.
2. Gently remove any jewelry, watches, belts, or constricting clothing from the injured area before it begins to swell.
3. When the burn is cooled, cover it with plastic wrap or place a clean plastic bag over a foot or hand. Apply the wrap lengthwise over the burn, not around the limb because the tissues swell. If you do not have plastic wrap or a plastic bag, use a sterile dressing or a gauze pad, bandaged loosely in place.
4. Take or send the casualty to the hospital if the casualty is a child, or if you are in any doubt about the casualty's condition.

SPINAL INJURY

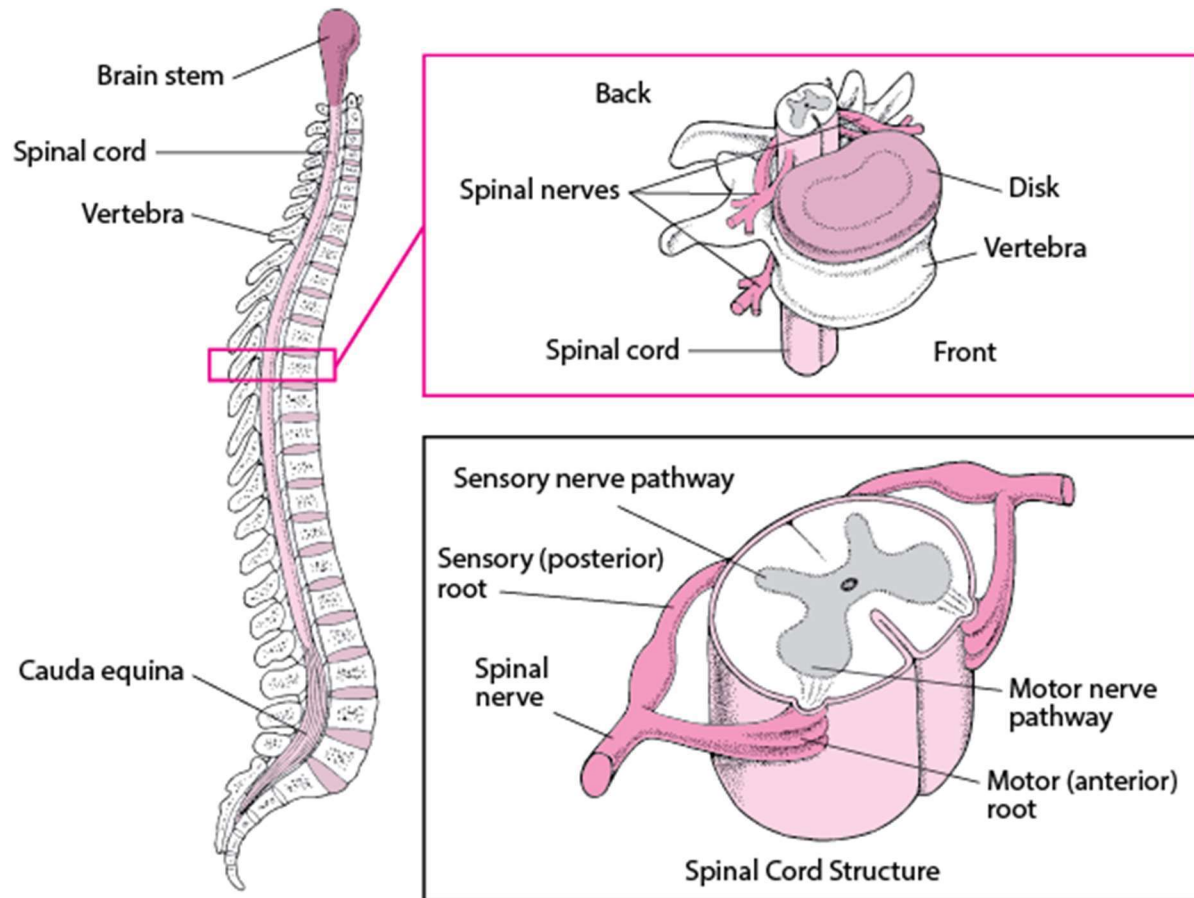
Injuries to the spine can involve one or more parts of the back and/or neck: the bones (vertebrae), the disks of tissue that separate the vertebrae, the surrounding muscles, and ligaments, or the spinal cord and the nerves that branch off from it.

The most serious risk associated with spinal injury is damage to the spinal cord. Such damage can cause loss of power and/or sensation below the injured area. The spinal cord or nerve roots can suffer temporary damage if they are pinched by displaced or dislocated disks, or by fragments of broken bone. If the cord is partly or completely severed, damage may be permanent.

Any of the following incidents should alert you to a possible spinal injury:

- Falling from a height, such as a ladder.
- Falling awkwardly, for instance, while doing gymnastics or bouncing on a trampoline.
- Diving into a shallow pool and hitting the bottom.
- Falling from a horse or motorcycle.
- Football tackle or misplaced hit.
- Sudden deceleration in a motor vehicle.
- A heavy object falling across the back.

- Injury to the head or the face



Recognition.

When the vertebrae are damaged, there may be:

- Pain in the neck or back at the injury site. This may be masked by other, more painful, injuries.
- Irregularity or twisting in the normal curve of the spine.
- Tenderness and/or bruising in the skin over the spine

When the spinal cord is damaged, there may be:

- Loss of control over limbs movement may be weak or absent.
- Loss of sensation, or abnormal sensations such as burning or tingling; a casualty may tell you that his limbs feel stiff, heavy, or clumsy.

- Loss of bladder and/or bowel control.
- Breathing difficulties.

Your Aim

- To prevent further injury.
- To arrange urgent removal to the hospital.

What to do for a conscious casualty



1. Reassure the casualty and advise him not to move. Call for emergency help, or ask a helper to do this.
2. Kneel or lie behind the casualty's head. Rest your elbows on the ground or on your knees to keep your arms steady. Grasp the sides of the casualty's head. Spread your fingers so that you do not cover his ears, so he can hear you. Steady and support his head in this neutral position, in which the head, neck, and spine are aligned.
3. Ask a helper to place rolled up blankets, towels, or items of clothing on either side of the casualty's head and neck, while you keep his head in the neutral position. Continue to support the casualty's head until emergency services take over, no matter how long this may be.

4. Get your helper to monitor and record vital signs—level of response, breathing, and pulse while waiting for help.

What to do for an unconscious casualty

1. Kneel or lie behind the casualty's head. Rest your elbows on the ground or on your knees to keep your arms steady. Grasp the sides of her head. Support her head so that her head, trunk, and legs are in a straight line.
2. Open the casualty's airway using the jaw-thrust technique. Place your fingertips at the angles of her jaw. Gently lift the jaw to open the airway. Take care not to tilt the casualty's neck.
3. Check the casualty's breathing. If she is breathing, continue to support her head. Call for emergency help or ask a helper to do this.
4. If the casualty is not breathing, begin CPR. If you need to turn the casualty, ***use the log-roll technique*** (below).



5. Monitor and record vital signs—level of response, breathing, and pulse—while waiting for help.